



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هبه احمد عبده دسوقي  
المحافظة : الشرقية  
الحى / المدينة : الحسينيه  
العنوان : ..  
رقم البطاقة : 28506041300201 رقم التليفون : 01113311016  
العمر (تاريخ الميلاد) : 04-06-1985 المهنة : ربه منزل  
تاريخ تسجيل الإستمارة : 20-09-2026

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (.....)  
المقرر بما فيه  
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي بيمبروليزوماب وريدي [14335] قسم العلاج : الاورام

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني  
الاسم: د/ندى حسن  
الاسم: د/نرمين رمضان  
الوظيفة: .....  
الوظيفة: .....  
الوظيفة: .....  
التوقيع: .....  
التوقيع: .....  
التوقيع: .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.  
تعتمد صحة البيانات المرفقة  
رئيس اللجنة ومدير  
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

..... /د. /..... /د. /..... /د.

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : .....  
قسم : .....  
جهة العلاج : .....  
نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

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|                                                                        |                                                                                     |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> هبه احمد عبده دسوقي<br><b>National ID:</b> 28506041300201 |  |
|------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                   |
|-------------------------------------------------------------------|
| المریضة تعاني من ورم غير حمید بالثدى و تحتاج الى Pembrolizumab IV |
|-------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

جمهورية مصر العربية  
بطاقة تحقيق الشخصية

هبة  
احمد عبده دسوقي  
١ م. ابو عامر  
مركز الحسينية - الشرقية

١٩٨٥/٠٧/٢٤ ٢٨٥٠٦٠٤١٣٠٠٢٠١

KQ9076240

١١١٣٣١١-١٦  
١١١٤٩٩٧٨٤٤

٢٠٢٤/٠٧ ٢٨٥٠٦٠٤١٣٠٠٢٠١  
ربة منزل  
أنثى  
مسلمة  
مقروجة  
خميس عبدالحميد حسين محمد  
البطاقة سارية حتى ٢٠٣١/٠٧/٢٣

١

|                 |                       |                 |                |
|-----------------|-----------------------|-----------------|----------------|
| Patient Name    | هبة احمد عيده         | Case No.        | 342927         |
| Sex/Age         | Female 41 Y           | Patient No.     | 24967          |
| Registered Date | 19/7/2026 01:14:26 PM | Referred By Dr. | Out Patient    |
| Validation Date | 12/8/2026 11:44:52 AM | Branch          | Gustave Roussy |
| Reporting Date  | 12/8/2026 12:07:32 PM | Pathology No.   | SR-826/26      |

### Breast Slide Revision

**Clinical Data:** Left breast 2 O'clock mass (BIRADs 5).

**Nature of Specimen:** Referred block & slide (Tru-cut biopsy).

### **Gross Examination:**

- One referred block labelled by (2325-26). Together with, one (H&E) stained slide.

### **Technique:**

- Sections prepared from paraffin block for immune-staining using Bench-Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

### **Microscopic Examination:**

- \* Histological type: Invasive duct carcinoma.
- \* Grade: 3 (differentiation 3, nuclear pleomorphism 3, mitosis 2).
- \* Ductal carcinoma in situ: Absent.
- \* Necrosis: Minimal.
- \* Angioinvasion: Absent.
- \* Perineural invasion: Absent.

### **IPT:**

- ER: Weak nuclear reaction in <1% of tumor cells (score 2/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 80%.

### **Diagnosis:**

**Left breast mass, Referred block & slide (Tru-cut biopsy):**

- Invasive duct carcinoma, Grade 3.



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة 5 - مصر  
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadema - Cairo, Egypt

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**IPT:**

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**\* \*Scoring system: "ER, PR"**

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

**\* \*\*Scoring system: "Her2"**

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

*Signature**Dr. Walaa Ibrahim Ghonaim, MD.*

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر  
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**GUSTAVE /  
ROUSSY**  
INTERNATIONAL  
LABORATORY**جوستاف /  
روسي**  
التراسونو  
مختبر**ECHO REPORT**

NAME :

عدد الحارة

DATE: 8 / 9/2026

ID NUMBER : 0170 / 07

SEX :

AGE:

|       |     |           |         |     |                   |
|-------|-----|-----------|---------|-----|-------------------|
| EF    | 60% | 55-75%    | PWT     | 1.0 | 0.6-1.1cm         |
| LVEDD | 4.6 | 3.5-5.8cm | AO.R    | 3.1 | 3-3.7cm           |
| LVEDS | 3.2 | 2-4cm     | LA      | 3.6 | 2-4cm             |
| FS    | 31% | 25-45%    | RV      |     | 0.7-2.3cm(supine) |
| SWT   | 1.0 | 0.6-1.1cm | AO.CUSP |     | 1.5-2.6cm         |

**LV Dimensions****LV Wall Thickness****LV Diastolic Function****SWMA****LA and Ao Root Dimensions****Mitral Valve****Aortic Valve****Tricuspid Valve****Pulmonary Valve****Right Side and Systolic Function****Intracardiac Masses and Effusion**

DR.

MHA Selectic changes

Kaz



Patient's Name:

Heba Ahmed Abdu Desouki

Date:

2026-07-21

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- No lung metastasis.

## **MULTI-SLICE CT SCAN OF THE ABDOMEN AND PELVIS AFTER I.V. CONTRAST ADMINISTRATION:-**

### **Findings:**

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

### **Opinion:**

- No hepatic or bone metastasis.

**Dr. Mai Shaaban MD**



Patient's Name: Heba Ahmed Abdu Desouki  
Date: 2026-07-21  
Referred by: Prof.Dr. no doctor  
Hospital: Goustave Roussey International Egypt

## **POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST WITH CORONAL RECONSTRUCTION:-**

### **Clinical History:**

- Left breast cancer.

### **Findings:**

- Left breast upper outer quadrant mass is seen measuring 5.3x5.5cm.
- Surrounding multiple satellites seen largest 1x1.5cm.
- Left breast edema and increased skin thickness.
- Left axillary enlarged LNs largest 3.5x3.3cm.
- Clear right breast and right axilla.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

### **OPINION:**

- Left breast upper outer quadrant mass.
- Surrounding multiple satellites.
- Left breast edema and increased skin thickness.
- Left axillary enlarged LNs.
- Clear right breast and right axilla.

|                 |                       |                 |                |
|-----------------|-----------------------|-----------------|----------------|
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- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67%: 80%.

### **Diagnosis:**

**Left breast mass, Referred block & slide (Tru-cut biopsy):**

- Invasive duct carcinoma, Grade 3.



|                 |                       |                 |                |
|-----------------|-----------------------|-----------------|----------------|
| Patient Name    | فيما احد عده          | Case No.        | 342927         |
| Sex/Age         | Female 41 Y           | Patient No.     | 24967          |
| Registered Date | 19/7/2026 01:14:26 PM | Referred By Dr. | Out Patient    |
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IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

*Signature*

**Dr. Walaa Ibrahim Ghonaim, MD.**



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Room No. (44) - First Floor-Building No. 987- Nile Corniche - Miter El Qandema - Cairo, Egypt

Name : Heba Ahmed Abdu Desouki      Date : 14-09-2026 08:27  
Branch : Dokki      Medical No. : 461115  
Ref.Physician Prof : Goustav Roussey Egypt      Ins.Company : تعاقد مستشفيات

## NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 41-year-old female, history of left breast carcinoma status post-surgical resection and chemotherapy, presenting with intermittent bone pain. Evaluation for skeletal metastasis.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

### Findings:

- Focal high-intensity radiotracer uptake isolated to the manubrium sterni.
- Remainder of the axial and appendicular skeleton demonstrates normal, symmetrical physiological tracer distribution.
- Prompt, symmetric renal clearance with expected soft tissue background.

### Impression:

- Solitary manubrial focal uptake in a patient with known breast carcinoma—highly suspicious for a solitary skeletal metastasis, though benign etiologies remain possible, for regional radiological correlation.
- No evidence of additional skeletal metastases.

Dr. Alaa Nagi MD Nuclear Medicine

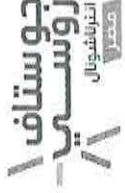
Medical No. : 9791

Patient's Name: heba ahmed abdo

Date: 2026-09-20

Referred by: Prof.Dr. no doctor

Hospital: Goustave Roussey International Egypt



## **Neck ultrasound examination**

### **Findings**

#### **Thyroid gland**

- Average size thyroid lobes& isthmus showing heterogeneous texture with echogenic bands, mild increase in vascularity
- Bilateral few scattered subcentimetric cystic nodules (TIRADS 1)
- Normal movement with swallowing
- Central trachea
- Intact carotid vessels
- No pathologically enlarged cervical lymph nodes
- **Opinion**

**Picture suggestive of thyroiditis for clinical& laboratory correlation with multiple cystic nodules (TIRADS 1)**

**DR: Nermeen Ramadan.MD**